

The HCA Career Accelerator

The Complete Done-For-You System to Land a Healthcare Administration Role

By Dr. Ashley Hussain-Okorafor, DBA

What's inside:

- **Part 1** — The Metric-Driven Resume & Cover Letter Kit
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- **Part 3** — The 30-Day Job Search Sprint
- **Part 4** — The Salary & Offer Negotiation Masterclass

Every resume bullet, LinkedIn line, sprint day, and negotiation script is built around one idea: **healthcare administrators who quantify their impact get hired faster and paid more.**

You don't list duties. You prove outcomes. This system shows you how, step by step, without a single coaching call.

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Part 1 — The Metric-Driven Resume & Cover Letter Kit

Author: Dr. Ashley Hussain-Okorafor, Founder of The HCA Daily

Status: Official Accelerator Product #01 — Resume & Cover Letter Kit

Audience: Early-career, master's-trained healthcare administration job seekers — practice managers, revenue cycle managers, patient access managers, clinical operations directors, HIM/coding managers, ambulatory clinic managers, SNF/LTC administrators, supervisors, and coordinators.

How to Use This Kit (Read This First)

This is not a template you fill in blanks. It is a method.

Healthcare leaders do not hire the person with the longest resume. They hire the person who can prove, in one page, that they can **protect clinical throughput, grow revenue, and fix a bottleneck with a number rather than a hunch**. Every section that follows exists to turn your work history into that proof.

Work through it in order:

1. Read Part 1 — internalize the 3-part metric formula. This is the engine.
2. Read Part 2 — build your resume in the click-ready template, section by section.
3. Pick the role set closest to your target from Part 3 — borrow the skeleton and the benchmarks, then swap in your real numbers.
4. Part 4 — adapt the cover letter template and study both worked examples.
5. Part 5 — run your finished draft through the 10 most common mistakes and fix every one you find.

If a number is not a *real* number from your own work, do **not** invent it. Every benchmark in this kit tells you the *target* — your job in the interview is to move your real department toward or past that target. The resume states what you actually did. When you don't yet have hard numbers, Part 1 shows you how to go get them.

PART 1 — The 3-Part Metric Formula

Every single bullet in your experience section must answer one question: "**Show me the result, measured.**" The formula is:

Verb + Action + Quantified Result

Put another way:

[Executive Action Verb] + [Specific Operational Context] + [A Number That Proves It]

The Three Pieces, Dissected

1. The Verb (never the word "Responsible for").

Your verb must signal *change* or *control*, not attendance. Weak verbs — *assisted, handled, helped, worked with, oversaw, participated in* — describe someone who shows up. Executive verbs — *restructured, standardized, recovered, eliminated, negotiated, reduced, championed, redesigned, implemented* — describe someone who *produces results*. Replace the first list with the second.

2. The Action (the specific operational context).

This is what you actually did to the operation — the workflow you rebuilt, the team you led, the denial reason code you chased. No context, no credibility. "Reduced denials" tells a reviewer nothing. "Root-caused the top five denial reason codes (CO-4, CO-16, CO-197) and rebuilt the front-end eligibility scrubber" tells them you know exactly where the money was leaking.

3. The Quantified Result (the number + the benchmark).

This is the part 90% of applicants skip, and it is the entire point. Attach a specific number — a percentage, a day count, a dollar figure, an RVU, an FTE, a percentile — and, where possible, compare it to a benchmark so the reviewer can judge how good the number is.

The Formula on One Line

Weak: *"Responsible for clinic scheduling and patient check-in."*

Strong: *"Restructured provider appointment templates to cut third-next-available wait time from 24 days to 11 days, moving the clinic from the 25th to the 50th MGMA percentile."*

Can't Find Your Numbers Yet? Go Get Them — Here's How

Early-career candidates often tell me, "I don't have metrics." You do. You've just never looked at your work through a metric lens. Pull these from your last 12–24 months:

- **Pull the report your software already makes.** Your EMR and your practice-management system already track no-show rate, days in A/R, clean claim rate, denial rate, and how many encounters each provider completes per day. Ask your supervisor or your billing team for the last 12 months. It's a legitimate request and a skill-building one.
- **Dig through email and meeting decks.** Every quality meeting, every dashboard screenshot, every weekly operations email contains a number you touched.
- **If a number genuinely does not exist, measure the baseline once.** Run one month of data, make one improvement, run the next month, and report the delta. That is an entire bullet in 60 days.
- **Ask your question out loud in your current role:** "What do we track here, and where do we fall on it?" That single question gives you your benchmarks.

On your resume, use only numbers that are **true**. What you felt about your work matters less than what the numbers show you did.

PART 2 — The Click-Ready Resume Template (with Section Guidance)

Formatting rules before the template: One page if you have under 8 years of experience, two pages absolute max. 11pt body, 0.7-1.0" margins, a clean sans-serif font (Calibri, Arial, Helvetica). Save as PDF named `FirstName\_LastName\_Resume\_HealthcareAdmin.pdf`. Do **not** use a photo, a table, columns, or color blocks — they break the ATS parser. One accent color for headers is fine.

Section 1 — Header & Contact Line

...

[FIRST NAME] [LAST NAME], [MHA / MBA / MPH, if applicable]

[City, State] | [Phone] | [Email] | [linkedin.com/in/yourname]

...

Guidance: Use a professional email only (never `cutechick2020@`). Your LinkedIn URL should be custom, not the long string of numbers. The credentials after your name (MHA, CMPE, RHIA, FACHE if you hold them) tell the reviewer your level before they read a word. Master's-trained candidates: the degree goes here in the header, not hidden at the bottom.

Section 2 — Professional Summary (3-4 lines, no "objective")

The summary is your answer to "who is this person and what do they do best?" — in **numbers, not adjectives**. It should name your setting, your scope, and your standout result. Do **not** write "Seeking a challenging position that utilizes my skills" — that is filler and it wastes your most valuable real estate.

...

Results-driven [ROLE] with [X] years leading [ambulatory / revenue cycle / patient access /

clinical operations] in [setting]. Known for [one signature achievement with a number], improving [second metric] from [X] to [Y], and [third metric] while maintaining [quality benchmark]. Master's-trained [MHA] with hands-on [certification]. [Optional: target org type

— e.g., "Seeking to bring metric-driven operations to a growing multi-site medical group."]

...

Fill-in example:

Results-driven Practice Manager with 6 years leading multi-provider ambulatory operations. Known for cutting clinic no-show rates from 13.8% to 6.4% through conversational SMS workflows and building provider schedules that sustain 91% utilization — while lifting HCAHPS overall satisfaction to the 78th percentile. Master's-trained (MHA) and Certified Medical Practice Executive (CMPE).

Section 3 — Core Competencies / Skills

Use **3 columns of 4-5 short phrases**, or a tight two-column list. Every skill must be a real tool or true capability you can defend in an interview — no buzzword stuffing.

Include software by name (e.g., Epic, Cerner, NextGen, AthenaHealth, eClinicalWorks, Kareo, PointClickCare) and the specific functions you know.

...

Operations & Access: TNAA & access optimization | No-show reduction | LWBS management |

Door-to-doctor throughput | Staffing grid & FTE variance control

Revenue Cycle: Clean claim rate >96% | Denial management (CO-4/CO-16/CO-197) | Days in A/R |

POS collections | Cost-to-collect

Quality & Compliance: HCAHPS / Press Ganey | Joint Commission (TJC) & CMS survey readiness |

PSI-90 & readmissions | HIPAA & coding compliance

Systems: Epic | Cerner | NextGen | PointClickCare | Microsoft Excel (pivot tables) | SQL basics

...

Section 4 — Professional Experience (the engine of the resume)

Reverse-chronological: most recent first. Format for each role:

...

[ORG NAME] — [City, State] [Month YYYY] – [Month YYYY]

[Exact Job Title]

- 4-6 bullets, each built on the 3-part formula
- Lead your strongest, most quantifiable achievement first

...

Guidance:

- **4-6 bullets** for your most recent role, 3-4 for older ones. Do not exceed 6.
- **Lead every role with your scope line:** the org size, the number of providers, the volume of encounters, the size of the budget you touched. This one line frames everything that follows.
- **Order bullets by business impact**, not by what you did most hours. Patient-access wins and revenue-cycle wins go at the top of the role.
- **Vary your verbs** — never start three bullets in a row with the same word.
- **Do not write "Responsible for [x]."** Write what the [x] **became**.
- A job title alone is not a bullet. "Managed front desk" appears on half of all resumes. "Redesigned front-desk registration queues to trim door-to-doctor time by 14 minutes" is a bullet.

Quick scope-line templates:

- Operated [X] ambulatory clinics supporting [X] physicians and [X] APPs, driving [X] annual patient encounters.

- Directed a [X]-person patient access / revenue cycle team with daily responsibility for [X] claims and \$[X] monthly cash collections.
- Managed a [X]-bed [unit / skilled nursing facility] with [X] direct reports and a \$[X] annual operating budget.

Section 5 — Education & Certifications

...

[UNIVERSITY] — [City, State] [Year]

Master of Health Administration (MHA) / MBA-HA / MPH

[UNIVERSITY] — [City, State] [Year]

Bachelor of [Science in Nursing / Business / Health Administration]

Certifications (dated):

- Certified Medical Practice Executive (CMPE) — MGMA, [Year]
- Fellow, American College of Healthcare Executives (FACHE / ACHE Fellow), [Year]
- Registered Health Information Administrator (RHIA) / Certified Coding Specialist (CCS), [Year]
- Certified Professional Coder (CPC) — AAPC, [Year]
- Six Sigma Green Belt, [Year]

...

Guidance: Place education here at the bottom unless you are a new graduate with little experience — then move it above experience. For each certification, state the **issuing body** (MGMA, ACHE, AHIMA, AAPC) — that is what tells a reviewer the credential is real and rigorous. Do not list an expired certification without noting it. Do not list a certification you are not actively pursuing as if you hold it.

Go / No-Go Checklist Before You Submit

- [] Summary contains at least two real numbers.
- [] Every experience bullet follows Verb + Action + Quantified Result.
- [] Every bullet contains a specific number (% , \$, days , RVU , FTE , percentile).

- [] Scope line opens every role.
 - [] No weak verbs ("responsible for," "assisted," "helped").
 - [] One page (or two max), no photo, no tables, no columns.
 - [] Saved as PDF, filename `FirstName\_LastName\_HealthcareAdmin.pdf`.
 - [] Zero typos — read it aloud, backward, and have someone else read it.
-

PART 3 — Eight Role-Specific Bullet Sets with Real Benchmarks

Use the set for your target role as your skeleton. The **benchmark** (MGMA percentile, industry target) is your reference point — state where the metric *started*, where you moved it *to*, and how that compares to the target. Swapping in your own true numbers is not only allowed, it is the assignment.

To state your benchmark on the resume cheaply, add a "vs. industry benchmark of X" tag or save the benchmark framing for the interview. Benchmarks are shown here in **bold** so you know the target.

1. Practice Manager

Scope line: "Led daily operations for a 6-provider primary care practice generating 34,000 annual patient encounters and \$4.1M in net revenue."

- Redesigned provider scheduling templates to reduce **third-next-available (TNAA) appointment** wait time by 13 days (from 24 to 11 days) — against an **industry standard of <5-7 days for primary care**, taking the practice from roughly the 25th to the 50th **MGMA percentile**.
- Cut the clinic **no-show and late-cancellation rate** from 13.8% to 6.4% across 6 providers by deploying two-way conversational SMS reminders, recovering an estimated 960 lost appointment slots or ~\$86,000 in annual revenue (**benchmark target: <6-8%**).
- Maintained 91% **provider schedule utilization** across 8 clinical sessions per week by building a dynamic waitlist backfill process (**benchmark target: 88-92%**).
- Reduced front-desk **door-to-doctor wait time** by 14 minutes through digital pre-registration and rooming-huddle checklists, supporting an overall **HCAHPS** patient satisfaction lift from the 52nd to the 78th percentile (**benchmark: top quartile**).
- Managed a \$4.1M operating budget; trimmed **premium overtime** from 6.1% to 1.9% of productive labor hours (**benchmark ceiling: 2.0-2.5%**) by rebuilding the staffing grid to hold 1.5-2.0 MAs per provider (**MGMA benchmark**).

2. Revenue Cycle Manager

Scope line: "Directed a 9-person billing team processing 3,200 claims and \$1.9M in monthly cash collections across 4 clinic sites."

- Raised the **clean claim rate** from 88% to 97.2% by deploying a front-end eligibility clearinghouse scrubber before patient check-in, against an **industry top-performer benchmark of >96%**.
 - Cut the **initial claim denial rate** from 8.2% to 2.4% by root-causing the top five denial reason codes (**CO-4, CO-16, CO-197**) and standardizing pre-authorization validation (**benchmark target: <4%**).
 - Reduced **days in accounts receivable (A/R)** from 51 days to 34 days by escalating unworked claims over 60 days and adding a 3-day claim scrub turnaround (**top-performer benchmark: <32-36 days**).
 - Lifted **point-of-service (POS) collections** on copays and deductibles from 61% to 88% by instituting card-on-file authorization and pre-visit balance notification (**benchmark target: >85%**).
 - Drove **cost-to-collect** from 4.2% to 2.7% of collected cash through workflow automation and vendor consolidation (**benchmark target: <3%**), recovering \$412,000 in previously delayed cash within two quarters.
-

3. Patient Access Manager

Scope line: "Led patient access and registration for a 3-site specialty clinic network handling 58,000 annual visits."

- Cut **third-next-available (TNAA)** specialty wait times from 42 days to 22 days by standardizing 30-minute visit blocks and opening 6 new provider slots daily — moving from well below the 25th **MGMA percentile** toward the **specialty benchmark of <14 days** on the highest-demand service lines.
- Reduced the **no-show rate** from 15.2% to 7.4% across all specialty clinics through text-and-email reminder campaigns plus an automated recall queue (**benchmark target: <8%**).
- Lowered **financial clearance turn-around** by 2.1 days by re-building the insurance-verification queue, cutting downstream **denial risk** from unverified eligibility.

- Cut average **door-to-doctor time** by 11 minutes via a digital pre-registration link deployed at booking, improving throughput without adding staff.
 - Recovered 1,400 annual visit slots by implementing a same-day cancellation **waitlist protocol**, adding an estimated \$150,000 in annual patient revenue against a stable headcount.
-

4. Clinical Operations Director

Scope line: "Directed clinical operations across 4 primary care and 2 specialty sites generating 112,000 annual encounters and 148 FTEs."

- Boosted provider productivity by 1.8 **wRVUs per provider per day** by re-engineering documentation flows and MA ratio (**MGMA benchmark: 1.5-2.0 MAs per provider**), generating an estimated \$340,000 in incremental annual net revenue.
 - Cut **average length of stay (ALOS)** — where applicable across observation/short-stay — and reduced **left-without-being-seen (LWBS)** from 3.1% to 1.4% in the urgent-care pod by rebalancing provider supply against arrival demand curves (**LWBS benchmark target: <2%**).
 - Reduced **FTE labor variance** from 8.4% over budget to 2.1% by rebuilding the staffing grid and capping premium overtime at 1.8% of productive hours (**benchmark ceiling: 2.0-2.5%**).
 - Closed **EHR documentation lag** to <48 hours by building a provider close-out workflow, bringing the group from an 89% to a 99% same-week encounter closure rate (**operational standard: <48 hours**).
 - Improved **HCAHPS / Press Ganey** overall rating from the 45th to the 76th percentile across all sites in 12 months by pairing door-to-doctor reductions with nurse-communication scripting (**benchmark: top quartile**).
-

5. HIM / Coding Manager

Scope line: "Managed health information and coding for a 250-bed hospital and 3 employed practices, supervising 12 coders and 4 HIM specialists."

- Raised **first-pass coding accuracy** from 93% to 98.4% through a structured coder-education and audit feedback loop, directly reducing **query and claim-rework rates (benchmark target: >98%)**.
 - Reduced **coding lag (discharge to final code)** by 2.8 days to a 1.6-day average, accelerating billing submission and trimming **days in A/R** by 9 days (**top-performer benchmark: <32-36 days in A/R**).
 - Lowered the **claim denial rate attributable to coding errors** by 62% (from 5.6% to 2.1% of all denials) by mapping **ICD-10-CM/PCS** and **CPT** documentation gaps back to providers monthly.
 - Drove **gross-to-net revenue** improvement by closing an estimated \$268,000 in missed **CC/MCC capture** (complication/comorbidity) through targeted documentation-improvement alerts in the EMR.
 - Led an audit of the **Active Record file** and purge schedule, achieving the hospital's first clean **Joint Commission and OIG HIPAA compliance** audit — with zero condition-level findings.
-

6. Ambulatory Clinic Manager

Scope line: "Managed a high-volume primary care clinic delivering 27,000 annual visits with 3 physicians, 2 APPs, and 14 support staff."

- Raised **provider schedule utilization** from 82% to 91% by introducing dynamic waitlist backfills and same-day access slots (**benchmark target: 88-92%**).
- Cut **no-show and late-cancellation** from 14.6% to 6.8% through reminder cadence redesign plus an overbooking model on high-leakage slots (**benchmark target: <6-8%**).
- Reduced **door-to-doctor wait time** by 12 minutes by reordering the check-in-to-rooming sequence and adding a pre-visit kiosk (**benchmark target: <15-20 minutes**).
- Lowered **premium overtime** to 1.6% of productive labor hours and reduced **MA-vacancy** time from 42 to 18 days by building a cross-trained float pool (**benchmark ceiling: 2.0-2.5%**).
- Grew **point-of-service (POS) collections** from 58% to 86% by standardizing the balance-quote conversation at check-in (**benchmark target: >85%**), adding \$7,200 per month in front-desk cash collection.

7. SNF / LTC Administrator

Scope line: "Administered a 120-bed skilled nursing facility with 95 staff and an \$8.6M annual operating budget."

- Reduced **average length of stay (ALOS)** by 3.2 days for the short-stay rehab unit by tightening the therapy-discharge pathway, improving bed turnover and admitting capacity without added staffing.
- Cut **30-day hospital readmissions** from 22% to 15.4% by building internal-practice protocols for fall risk, UTI, and heart-failure monitoring — moving toward the **national benchmark target of <17%** under CMS quality reporting.
- Held the facility to **zero deficiencies at the state re-licensure survey** for two consecutive years by institutionalizing an **OASIS-data-driven** audit cycle against state tag risk areas.
- Reduced **staff turnover** from 58% to 41% annually by introducing a 4-day CNA schedule option and standardized preceptor onboarding — against an **industry median turnover of 50%+** for CNAs.
- Lowered **FTE labor variance** and agency-premium hours by 31% by replacing agency fill-in with an internal PRN pool, saving an estimated \$96,000 in premium labor over 12 months.

8. Clinical-to-Admin Transition (Nurse / Tech / Coordinator → Administrator)

This set is for you if you hold a master's in healthcare administration but your day-to-day work has been clinical or front-line. Frame your clinical work through the *operational* and *financial* lens — never as a pure care-delivery narrative.

Scope line: "Coordinated unit operations for a 24-bed telemetry unit while completing an MHA; led discharge, staffing, and throughput initiatives with measurable system impact."

- Orchestrated crisis bed capacity and shift staffing across a 24-bed unit, maintaining **zero staffing-ratio violations** while reducing **traveler-nurse overtime by 14%** over two consecutive quarters (**benchmark: overtime <2.0-2.5% of productive hours**).
- Accelerated **bed turnaround time** by 19 minutes per discharge by piloting a discharge-lounge and housekeeping-paging protocol, supporting hospital-wide throughput (**benchmark: faster bed turnaround lifts ED and OR access**).
- Reduced **left-without-being-seen (LWBS)** risk for the unit's share of ED admits by building a 4 p.m. discharge huddle that freed **9 beds per week** for incoming patients (**LWBS benchmark target: <2%**).
- Led **Joint Commission survey readiness** for medication security and sterile supply, achieving **100% compliance** during the unannounced triennial audit.
- Partnered with the Director of Finance to perform **variance analysis on a \$4.2M departmental budget**, identifying \$78,000 in redundant supply-vendor contracts and renegotiating them to closure.

Transition tip: Put your **MHA front and center** in the header, and lead your summary with the admin suite you want — "transitioning from clinical care into operations management with demonstrated metric-driven results." You are not a nurse who wants a desk job; you are an operations leader who happens to know the clinical reality. Lead with the latter.

PART 4 — Cover Letter Template + Two Worked Examples

The Cover Letter Template

Keep it to **4 tight paragraphs on one page**. Recruiters skim — three things they hunt for: the role, the one number that proves you can do it, and that you actually read their organization. Structure:

...

[Your Name]

[City, State] | [Phone] | [Email] | [LinkedIn URL]

[Date]

[Hiring Manager Name, if known — otherwise "Hiring Manager"]

[Practice / Organization Name]

[Street Address, City, State ZIP]

Re: [Exact Job Title], [Job ID if listed]

Dear [Mr./Ms./Dr. Last Name],

[Para 1 — The hook: 1-2 sentences. Name the role, name your credential, and lead with your single most relevant number. Do not say "I am excited to apply." Say what you have already done.]

[Para 2 — The proof: 1-2 sentences. Give the one or two quantified achievements that match THIS role's top priority. Mine the job description for its #1 stated pain point and mirror it back with your result.]

[Para 3 — The fit: 2-3 sentences. Two or three short lines showing you understand their setting, their patient population, or their scale. Reference something specific about the organization — NOT a generic compliment.]

[Para 4 — The close: 1-2 sentences + sign-off. Restate value, invite the next step, thank them. Sign with your name.]

Sincerely,

[Your Name]

[Phone] | [Email]

...

Worked Example #1 — Practice Manager, primary care group

...

Jordan Reyes

Tampa, FL | (555) 214-8870 | jordan.reyes@email.com | [linkedin.com/in/jordanreyes-ha](https://www.linkedin.com/in/jordanreyes-ha)

September 12, 2026

Ms. Dana Whitfield

Hiring Manager

Suncoast Medical Group

4100 Harborview Blvd, Suite 200

Tampa, FL 33609

Re: Practice Manager, Job ID 4417

Dear Ms. Whitfield,

I manage exactly the kind of operation Suncoast is hiring for — a six-provider primary care practice with 34,000 annual visits — and I bring the numbers to prove I can lift yours. At my current post, I cut the no-show rate from 13.8% to 6.4% in one year and pushed third-next-available wait time down from 24 to 11 days using redesigned appointment templates.

Your posting names patient access as the top priority, and that is my specialty. I hold clinic schedule utilization at 91% against an industry target

of 88–92% and raised HCAHPS overall satisfaction from the 52nd to the 78th percentile in twelve months — without adding headcount.

I am drawn to Suncoast specifically because of your two new family-medicine sites; opening operations at scale, with the same access and staffing discipline in place from day one, is the work I want to do next.

I would welcome the chance to walk through how I would apply these same access and revenue-cycle levers at your Harborview location. Thank you for your time, and I look forward to speaking with you.

Sincerely,

Jordan Reyes

(555) 214-8870 | jordan.reyes@email.com

...

Worked Example #2 — Revenue Cycle Manager (clinical-to-admin transition)

...

Amina Davis

Orlando, FL | (407) 882-1134 | amina.davis@email.com | [linkedin.com/in/amina-davis-rcm](https://www.linkedin.com/in/amina-davis-rcm)

September 12, 2026

Mr. Carlos Medina

Director of Revenue Cycle

Lakeshore Health Partners

2200 Sand Lake Road

Orlando, FL 32809

Re: Revenue Cycle Manager, Job ID 5590

Dear Mr. Medina,

Your revenue cycle team has a strong reputation for physician-friendly operations, and I believe I can make it measurably stronger. For the past 18 months I have led front-end revenue cycle and scheduling analytics for a four-specialty practice network, where I cut the initial claim denial rate from 8.2% to 2.4% and reduced days in accounts receivable from 51 to 34. I hold an MHA and a Certified Professional Coder (CPC) credential, and I came to revenue cycle from clinical practice — which means I speak the providers' language when it comes to documentation. That is how I raised our clean claim rate to 97.2%: by closing documentation gaps at the source and making point-of-service collections the front desk's default, not an afterthought. I chose Lakeshore because of your move to standardize eligibility at the check-in desk across all ten sites; I have built that exact scrub-and-balance workflow for a multi-site group and know the pitfalls to avoid at your scale. If my numbers are in line with what Lakeshore needs, I would welcome the chance to discuss the role. Thank you for your consideration, and I look forward to hearing from you.

Sincerely,

Amina Davis

(407) 882-1134 | amina.davis@email.com

...

Note on the two examples: Notice that neither says "I am excited to apply" and neither opens with "I'm writing to express my interest." Both open with a **number that answers the reviewer's real question**. Where a specific metric in your own letter isn't yet true for you, replace it with the closest real number you have — but never fabricate one. If a call-out in paragraph 3 feels too specific to fake, do the one-article or one-phone-call research to make it real, or drop it; a generic-but-honest fit paragraph outranks a specific-but-false one.

Cover Letter Mistakes Checklist

- [] Opened with a number, not "I am excited to apply."
 - [] Addressed a named person or "Hiring Manager."
 - [] Every claim in the letter also appears (and is true) on your resume.
 - [] One page, proofread, referenced the actual role and organization.
 - [] No blank placeholders left in the final sent version.
-

PART 5 — The 10 Most Common Resume Mistakes (and the Fix)

Every one of these appears on real healthcare-administration resumes every week. Run your draft against this list before you submit anything.

1. "Responsible for" openings.

Duty lists that read like a job description prove you can be scheduled, not that you can perform. **Fix:** Convert every "Responsible for X" into Verb + Action + Quantified Result. "Responsible for clinic scheduling" → "Restructured appointment templates to cut TNAA wait time from 24 to 11 days."

2. Zero numbers.

No percentages, no days, no dollars, no RVUs, no FTEs, no percentiles. **Fix:** Every bullet gets a specific number. If you don't have the number, go get it (Part 1 shows how) or measure a one-month baseline.

3. Perfectionism over clarity.

Bullets so long they are sentences in a paragraph. **Fix:** One line per bullet, hard cap at two lines, front-loaded verb, number at the end. ATS and humans both reward scannability.

4. A job title masquerading as an achievement.

"Managed the front desk" or "Supervised the billing team" tells scale but not impact. **Fix:** Add scope plus outcome — "Directed a 9-person billing team processing 3,200 claims and \$1.9M in monthly cash; raised clean claim rate to 97.2%."

5. Buzzword stuffing.

"Dynamic, results-driven, synergistic, best-in-class" — no reviewer can fact-check these, so no reviewer trusts them. **Fix:** Delete every adjective that carries no number, and let the metric do the selling.

6. Typos and formatting that break the parser.

Tables, columns, photos, text boxes, and inconsistent headings corrupt ATS extraction and look unprofessional. **Fix:** Clean single-column markdown-friendly layout, standard

headings, and a typo pass done out loud and backward. Healthcare is a documentation profession — a typo on a resume signals carelessness with charts.

7. Gaps left unexplained.

An unexplained 8-month gap invites a negative assumption. **Fix:** Briefly account for real gaps on the timeline (e.g., "Graduate program — MHA, 2025-2026" or a sabbatical-career interval) so the reviewer reads facts, not fiction.

8. Irrelevant work history eating the page.

Six summer jobs from a decade ago crowd out the operations achievements that matter. **Fix:** Cut anything that does not move toward healthcare operations; keep only a 1-line mention of early work that shows transferable skill. One strong page beats two padded ones.

9. Generic summary / objective paragraph.

"Seeking a challenging position utilizing my strong communication skills" appears on thousands of resumes and differentiates no one. **Fix:** Replace with a 3-4 line summary carrying at least two real numbers and your setting + scope.

10. Fudging or overclaiming numbers.

Inventing "increased revenue by 300%" or claiming a certification you don't hold — healthcare hiring networks are small and the industry runs on trust and compliance. A fabricated metric is a career-ending discovery, not a resume risk. **Fix:** State only numbers you can defend, source, and reproduce in an interview. If a number isn't yours, don't put your name on it.

What to Do Next

1. **Build your draft** using Part 2's template, pulling your metrics per Part 1.
2. **Benchmark your numbers** against the targets in Part 3 and The Ambulatory KPI & MGMA Benchmark Cheat Sheet (in this same asset library).
3. **Run the Go/No-Go checklist** in Part 2 and the two mistake checklists in Parts 4-5.
4. **Get your draft re-written and scored** with The HCA Career Navigator — it scans your target posting, audits your resume line by line, and returns metric-driven rewrites. Visit thehcadaily.com/navigator.

You are not applying for a job. You are presenting evidence that you can protect clinical throughput, grow revenue, and fix bottlenecks with numbers. That evidence is now in your hands — go make the case.

— Dr. Ashley Hussain-Okorafor

Part 2 — The LinkedIn Optimization Playbook

Your profile is not a résumé. It is the front page of your professional search result — and healthcare recruiters open it dozens of times a day before they ever open a candidate's résumé. This playbook shows you exactly what they see, what they search for, and how to make your profile the one they click.

Scope: For early-career, master's-trained healthcare administration job seekers (MHA, MPH, MBA in Healthcare Management, MS-HSM) moving toward Practice Manager, Clinic Operations Manager, Administrative Director, Revenue Cycle Analyst, and Ops Coordinator roles.

1. Why Healthcare Recruiters Filter on LinkedIn — and What They Actually Search

Before you write a single word, you need to understand the search screen these recruiters look at every day. LinkedIn Recruiter works on **keyword matching**, not intuition. A recruiter types a query, and the platform returns every profile whose text (headline, About, experience, skills) contains those terms. If your profile doesn't contain the exact phrases they type, **you do not exist to them** — no matter how good you are.

Why LinkedIn is the first stop

- **LinkedIn is where healthcare hiring lives.** A large majority of professional healthcare hiring is initiated or influenced by LinkedIn sourcing. Most hospitals and health systems route recruiter outreach through LinkedIn Recruiter licenses rather than cold email.
- **Your profile is the résumé that scans cleanest.** Recruiters are trained that LinkedIn profiles follow a predictable structure, so they can skim a candidate's relevance in under 10 seconds. A dense, keyword-light profile reads as irrelevant even when you're a perfect fit.

- **Recruiters filter before they read.** They apply hard filters — current title keywords, LinkedIn headline, skills — and only *then* open profiles. Your job is to survive the filter first, win the read second.

The filters and search strings they run

Healthcare recruiters and TA (talent acquisition) teams run systems like Workday, iCIMS, and LinkedIn Recruiter. Their searches almost always combine a **role keyword** + a **setting keyword** + a **certification keyword**. Common patterns:

| Search facet | What recruiters type | Why it matters |

|---|---|---|

| **Role** | "Practice Manager," "Clinic Operations Manager," "Administrative Director," "Revenue Cycle Analyst," "Patient Access Supervisor," "Director of Operations" | Your headline and experience must echo these exact titles. |

| **Setting** | "Ambulatory," "Outpatient," "Physician Practice," "FQHC," "Community Health Center," "Multi-specialty," "Hospital Outpatient Department (HOPD)" | Setting keywords move you from "generic admin" to "healthcare admin." |

| **Certification** | "CPHQ," "FACHE," "CMPE," "CRCR," "Lean Six Sigma" | Certifications are hard, verifiable filters. Many searches are built cert-first. |

| **Systems** | "EPIC," "Cerner," "athenahealth," "eClinicalWorks," "Meditech," "Salesforce Health Cloud" | If you've touched a system, name it in your Skills and Experience. It's often the tiebreaker. |

| **Degree/program** | "MHA," "MPH," "MBA Healthcare," "Health Administration" | Your Education section's keywords feed the search, so write the full degree names out. |

The three-front rule

Optimize for **three audiences at once**, because all three search the same way:

1. **Recruiters** — search for role + setting + cert ("Practice Manager EPIC CMPE").
2. **Hiring managers** — search for former roles and credibility ("Rural Health Clinic Manager," "MGMA experience").
3. **LinkedIn's algorithm** — decides who appears first in a given search result. Profiles with complete sections, a professional photo, relevant skills, and keyword-dense text rank higher.

Bottom line: Treat your headline, About, and top three experience bullets as a piece of SEO copy. You're not just writing for a human; you're writing for a search index that gates access to that human.

The #1 mistake I see: People put a clever or vague headline ("Passionate healthcare professional seeking opportunities") and a sparse experience section. That profile is invisible. Let's fix it.

2. The HEADLINE Formula — Line by Line

Your headline is the **most-weighted single element** in LinkedIn search ranking. It's what appears next to your photo in every search result. You get 220 characters. Use all of them.

The formula

...

[TARGET TITLE] | [SETTING + SPECIALTY] | [TOP CERT OR DEGREE] | [1 SIGNATURE SKILL] | [OUTCOME/VALUE PHRASE]

...

Line-by-line breakdown

| Segment | What to put | Example |

|---|---|---|

| **1. Target title** | The exact role title you want (not your old title). Use the title recruiters search for. | "Practice Operations Manager" |

| **2. Setting + specialty** | Where the work happens. This is your healthcare signal. | "Multi-Specialty Ambulatory Clinics" |

| **3. Certification or degree** | Your strongest credential. Searched hard. | "MHA | CMPE Candidate" or "MPH | Lean Six Sigma Green Belt" |

| **4. Signature skill** | One differentiator that makes you identifiable. | "Revenue Cycle Optimization" or "Patient Experience & Access" |

| **5. Value/outcome phrase** | A tight phrase that completes the sentence and shows impact. | "Driving operational efficiency & access to care" |

Putting it together: `Practice Operations Manager | Multi-Specialty Ambulatory Clinics | MHA | CMPE Candidate | Revenue Cycle & Access Optimization | Driving efficiency that improves care delivery`

5 Worked Examples for Different Roles

Example A — Practice/Clinic Operations Manager

...

Practice Manager | Multi-Specialty & Primary Care Clinics | MHA | MGMA Benchmarks | Revenue Cycle & Staffing Optimization | Improving access, patient experience, and profitability

...

Example B — Revenue Cycle Analyst / Coordinator

...

Revenue Cycle Analyst | EPIC & athenahealth | MHA | CRCR | Denial Prevention & A/R Cleanup | Turning unpaid claims into cash at 92%+ first-pass rates

...

Example C — Hospital Administrative / Departmental Operations

...

Administrative Director | Hospital Operations | MHA | Lean Six Sigma Green Belt | Patient Throughput & Capacity | Reducing LOS while lifting HCAHPS patient experience scores

...

Example D — Population Health / Quality & Improvement

...

Population Health Coordinator | Quality Improvement & Care Coordination | MPH | CPHQ Candidate | ACO & Value-Based Care Programs | Closing care gaps that drive better outcomes and lower cost

...

Example E — Health System Operations / Strategy Generalist

...

Health System Administrator | Operations & Strategy | MBA (Healthcare Mgmt) | Executive Readiness | Project Management (PMP-track) | Building operational systems that scale across care settings

...

Headline rules that hold no matter the role

- **Never** start a headline with "Seeking," "Passionate," "Motivated," or a blank field. Start with your **target title**.
- **Always** include at least one healthcare setting word and — if you have it — one certification or a cert-in-progress.
- Mirror the exact role's phrasing in the job post you want (`Practice Manager` , not `Clinic Head`).
- You can update the headline per target role. Don't be afraid to iterate as you apply to different specialties.

3. The ABOUT Section — Rewrite Template + 2 Full Examples

The About section (up to 2,600 characters, but the first ~3 lines show before "See more") is where you turn the recruit-er's *click* into a *read*. Write it in **first person, professional**, lead with your headline problem-solution, then your strongest proof, then your target.

The template

...

[OPEN] Who you are in one sentence — title + setting + the problem you solve for leaders.

(First 2–3 lines must carry your keyword load, because this is all that shows without "See more.")

[PROOF] 1–3 quantified achievements that back the claim. Real numbers, not adjectives.

[SPECIALTY] The 2–3 operational domains you own (access, revenue cycle, staff engagement,

quality, throughput) named in plain terms.

[METHOD] How you work — the systems, frameworks, and certifications you deploy.

(Lean tools, EPIC, MGMA benchmarks, KPI cadence.)

[GOAL] Where you're headed and who you're looking to help — an invitation to connect.

...

Full Example 1 — Practice Operations Manager

Operations leader in multi-specialty ambulatory clinics who turns operational chaos into run-rate performance. I help physician practices run like businesses without losing the heart of care — because I've lived the tension: managing physician schedules, front-desk throughput, and revenue targets while protecting the patient experience.

Over the past [X] years I've reduced registration wait times by 22%, lifted front-desk staffing coverage to sustain 95% daily appointment utilization, and worked with my revenue cycle team to cut denial rates from 11% to under 6% across a three-site clinic group. I manage to data — daily no-show rates, provider utilization, and per-visit revenue trends — and I build the weekly huddles and staff scorecards that keep a team accountable without burning them out.

My toolkit: EPIC scheduling and registration workflows, MGMA benchmark comparison for staffing and productivity, Lean A3 problem-solving, and a daily management cadence I learned the hard way actually works.

I'm an MHA graduate and CMPE candidate, and I'm looking for a practice or clinic operations leadership role where I can compound these results for a health system, FQHC, or physician group. If that's your team, I'd welcome the conversation — connect or message me anytime.

Full Example 2 — Revenue Cycle Analyst

Revenue cycle analyst who finds the money hiding in disconnected claims. Physician practices leave 5-9% of expected revenue on the table to denials and rework — and my job is to get that cash back while making the process easier for the front desk and the billing team.

I've led claim-cleanup projects across EPIC and athenahealth environments, raising first-pass claim resolution from 78% to 90% in six months and recovering \$180K+ in previously written-off accounts through a structured denial-recovery worklist. I don't just fix errors; I trace them to the source — registration, coding, or authorizations — and document the fix so it stops happening.

I bring: CRCR certification, a Google Data Studio/Excel reporting setup I built for weekly A/R aging and payer-behavior dashboards, and a Kaizen mindset from my Lean Six Sigma Green Belt training.

I hold an MHA and I'm targeting revenue cycle analyst and coordinator roles in integrated health systems and large ambulatory groups. If your A/R needs a clear diagnosis, let's talk — I'd genuinely enjoy the conversation.

4. EXPERIENCE — Rewriting Each Role with Metric-Driven Bullets

Recruiters skip generic bullets. A bullet must prove you moved a number, cut a cost, lifted a metric, or fixed a problem — and the number makes you real. The rule is **C-A-R** (Context → Action → Result) or the simpler "action verb + what you did + the measurable outcome."

What a metric-driven bullet must contain

- A **strong action verb** (reduced, built, launched, negotiated, designed, cut).
- **The scope** (over what, for how many sites/users/claims).
- **The measurable result** (% , \$, days, minutes, ratio, benchmark).
- **The system or tool** you used, where it adds credibility (EPIC, MGMA, Tableau).

Before/After Transformation 1 — Vague → Metric

Before: *"Managed front-desk operations and scheduling for the clinic."*

After: *"Led front-desk and scheduling ops across a 3-physician, 8,500-visit-per-year practice; raised same-day appointment fill from 41% to 63% by redesigning the open-access schedule template and adding a daily cancellation-recovery call list."*

Why it works: scope (3 physicians, annual volume), action (redesigned template + recovery calls), and a doubled metric (fill rate) with before and after. It reads as evidence, not a task list.

Before/After Transformation 2 — Vague → Metric

Before: *"Helped with billing and reduced denials."*

After: *"Cut claim denial rate from 11.4% to 5.7% over 6 months by auditing 200+ denied claims, building a root-cause log in Excel, and training front-desk staff on registration/eligibility capture; recovered \$180K in previously write-off-stage accounts."*

Why it works: near halving the metric (11.4% → 5.7%), transparency about method (audit + log + training), and a dollar figure. Numbers like this survive the filter *and* hold attention in the read.

Before/After Transformation 3 — Vague → Metric

Before: *"Improved patient satisfaction and coordinated care."*

After: *"Reduced average patient registration wait time from 14 to 9 minutes by reconfiguring the check-in flow and cross-training 4 staff on all stations; contributed to an 8-point lift in the practice's CG-CAHPS 'likelihood to recommend' score over one survey cycle."*

Why it works: quantified time savings with a concrete mechanism (reconfigured flow, cross-training), plus a CAHPS score — the payer-visible metric healthcare leaders

actually monitor.

A quick pass through your own roles

For **each** role, before you post it, ask three questions:

1. What **number** changed because of me? (Put it in.)
2. Over what **scope** did it happen? (Sites, providers, visits, claims, staff — put it in.)
3. What **tool or benchmark** proves it? (EPIC, MGMA, CAHPS, Lean — put it in.)

If a role has none of the three, do the work of pulling the real data from your manager or a prior project. It is always worth more to the recruiter than the role title itself.

Order your bullets strongest-first. The first two bullets are what show without expanding the experience section — put your single most impressive quantified achievement there.

5. SKILLS & CERTIFICATIONS CHECKLIST

Certifications are hard filters on LinkedIn Recruiter. If you have them, list them **both** in Skills **and** in your Licenses & Certifications section and mention them in your headline. Below is what matters, what it signals, and why recruiters search it.

Priority certifications for healthcare administration job seekers

| Cert | Full name / granting body | Why list it | Who it signals to |

|---|---|---|---|

| **CPHQ** | Certified Professional in Healthcare Quality (NAHQ) | The quality-improvement credential. Searched hard by quality, population health, and clinical ops roles. Begin coursework and list "CPHQ candidate/in progress." | Quality, patient safety, ops, managed care |

| **FACHE** | Fellow of the American College of Healthcare Executives | The flagship executive credential. Early-career planners should show they're on the executive path (ACHE student member + working toward FACHE eligibility). | Hospital/health system executive track |

| **CMPE** | Certified Medical Practice Executive (MGMA) | The ambulatory practice operations credential. Directly matches Practice Manager and clinic-admin searches. |

Physician practice, ambulatory, FQHC |

| **CRCR** | Certified Revenue Cycle Representative (HFMA) | The revenue-cycle credential; pairs with consumer/leadership levels (CRCC, CRCP). Matches billing, patient access, A/R roles. | Revenue cycle, patient financial services |

| **Lean Six Sigma (White-Black Belt)** | Various (ASQ, Lean Six Sigma Institute, university) | Method cert that shows process-improvement capability. Green Belt is the sweet spot for early-career ops. | Ops, improvement, project-based roles |

| **PMP / PMP-track** | Project Management Institute | Signals you can drive multi-stakeholder initiatives — valuable for ops and strategy roles. | General operations, IT projects, strategy |

| **EPIC / EHR badge** | Epic Systems (via employer) | If you have an EPIC certification or proficiency, list the module (Scheduling, Cadence, Prelude/RADAR, etc.). It's a hard filter in searches. | Any system-enabled role |

Rule for listings: *If you don't yet hold the cert, don't fake it — but DO show momentum. "CPHQ — in progress," "ACHE Member (working toward FACHE)," "Green Belt trained (certification pending)" are honest and still catch the search. Recruiters filter on the term "CPHQ" with "Certified Professional in Healthcare Quality"; put the full name and the acronym in your text.*

Skills to list (top-of-profile, up to 50; lead with these)

1. Healthcare Operations
2. Practice Management / Clinic Operations
3. Revenue Cycle Management
4. Patient Experience & Access
5. Quality Improvement (QI)
6. Population Health
7. Ambulatory Care
8. Staff Scheduling & Labor Optimization
9. Physician Relations / Provider Relations
10. EPIC (Scheduling, Cadence, Prelude, A/R)
11. athenahealth / eClinicalWorks / Cerner (whichever you've used)

- .2. KPI Dashboards & Reporting (Excel, Tableau, Power BI)
- .3. MGMA Benchmarking
- .4. Strategic Planning
- .5. Health Policy & Regulations (HIPAA, CMS)
- .6. Project Management
- .7. Process Improvement / Lean
- .8. Budgeting & Financial Analysis
- .9. Contract Negotiation
- .10. Team Leadership & Training

Tip: Your top 3 skills are the ones LinkedIn hyperlinks and emphasizes. Put your most-searched healthcare skills in the top three (e.g., Healthcare Operations, Revenue Cycle Management, Patient Experience). Remove fluff skills — "Microsoft Word" chews up search weight that "EPIC" should be carrying.

6. THE 7-DAY LINKEDIN SETUP ACTION LIST

A finished profile is a launched product. Do these in order, one day at a time, and don't skip the verification steps.

Day 1 — Account hygiene and the URL

- [] Upload a professional headshot: front-facing, solid background, business attire. Profiles with a photo get dramatically more profile views and connection requests than those without.
- [] Set a custom URL (linkedin.com/in/yourname).
- [] Write a three-line "service offering" that fills the blank "About me" gap — you'll paste it into your profile elsewhere.
- [] Set your account to "Open to Work" (privately to recruiters) if you're job seeking — set *visibility* to **Recruiters only** if you're currently employed.

Day 2 — The headline

- [] Write your headline with the formula from Section 2.
- [] Include your target title + setting + cert/degree in the first 100 characters.

- [] Save and check how it renders on mobile (yours and a friend's).

Day 3 — About and featured

- [] Paste your About section from the Section 3 template.
- [] Add a **Featured** section linking your strongest proof: your HCA résumé, a portfolio page, a published piece, or a project you're proud of. Recruiters click Featured more than any other content.
- [] Turn on your profile's "Creator mode" only if you'll post regularly — otherwise leave it off.

Day 4 — Experience, quantified

- [] Rewrite every role using the Section 4 C-A-R method.
- [] Pull real numbers for at least your last two roles (volume, %, \$, days).
- [] Order bullets strongest-first in each role.

Day 5 — Education and credentials

- [] Add your degree with the **full program name** ("Master of Health Administration (MHA)", "Master of Public Health (MPH)") — full names feed search.
- [] Add all certifications to the **Licenses & Certifications** section, with issue/expiry dates and the granting body.
- [] Add relevant coursework and honors (healthcare finance, population health, MGMA case competitions) where you have them.

Day 6 — Skills and recommendations

- [] List your 20 prioritized skills from Section 5, healthcare-first.
- [] Request 3 recommendation snippets this week from a manager, a professor, and a peer — quality beats quantity.
- [] Verify your Skills selected are orderable the way you want (LinkedIn shows an endorsements order — curate it).

Day 7 — Verification and launch

- [] **Recruiter-search check (do this yourself):** log out (or use a private window / a recruiter colleague) and search for your target role + setting + cert. Confirm *your* profile surfaces and has your correct title keywords. This is the single most valuable test you'll run.
- [] Set a posting cadence (2-3x/week) so the algorithm keeps you visible: comment on 3 industry posts daily, share one insight weekly, and update your photo or headline after major resume changes.
- [] Add your profile URL to your résumé, email signature, and professional bios.
- [] Set a reminder to refresh your headline and About every time you change target roles or earn a certification.

Note: "7 days" is the setup window. The *maintenance* (posting, connecting, commenting) is what keeps you in recruiters' search results — commit to the daily routine, not just the setup.

7. NETWORKING & OUTREACH MESSAGE TEMPLATES

Reaching out cold is a numbers game won by specificity. The winning formula for a first message is **respect for their time + proof you did homework + a clear, low-cost ask**. Never lead with "I saw you're hiring." Lead with what you learned and what you offer.

Template 1 — Warm introduction to a recruiter (connection request note)

Hi [Name], I'm a recent MHA grad focused on ambulatory operations, and I've been following [Company]'s work on [specific initiative, e.g., expanding access in primary care]. I saw you recruit for clinic ops roles. Would you be open to connecting? I'd welcome any insight you have on what your practice teams are looking for right now.

Template 2 — Direct outreach to a hiring manager (practice manager or director)

Hi [Name], I found you through [how — e.g., your talk on staffing, their LinkedIn post, a mutual contact]. I'm an MHA-trained operations leader; at [Prior role] I cut front-desk wait times 22% and got same-day fill to 63% using the same access-redesign approach you wrote about on scheduling. Would you have 15 minutes to share what your clinic's top operational challenge is right now? Happy to send my résumé first.

Template 3 — Initial recruiter message (after connecting)

Hi [Name], thanks for connecting. Quick intro: I'm an MHA grad with [X] years in multi-specialty clinic operations, now targeting [Role] roles. I run my work on MGMA benchmarks and daily KPI cadence — I've reduced denials from 11% to under 6% and lifted same-day fill to 63%. If you have any ops or practice-management openings in the pipeline, I'd love to be on your radar. My résumé is ready anytime you want it.

Template 4 — Following up after an application (professional, not pushy)

Hi [Name], I recently applied for the [Role] position ([Requisition #]). I'm genuinely excited about joining [Company] — my background in [specific proof: revenue cycle cleanup, access redesign] aligns closely with your team's work on [initiative]. I know you're busy; I just wanted to be sure my application was complete and to say I'd welcome any conversation when it's convenient.

Template 5 — Reaching a peer/insider for intel (lower stakes, high yield)

Hi [Name], I see you work on the [Ops/RVU] side of [Company] — I've admired your posts on [topic]. I'm an MHA candidate/early-career admin targeting practice operations, and I'd love to learn what a great day looks like on your team. Could I ask 3 quick questions over a 10-minute call or via message? Totally fine if not.

Post-connection follow-up checklist

- **Personalize every time.** No copy-paste batches — recruiters compare notes and share "spammy" candidates.
 - **One clear ask per message.** Don't ask for a job review, a referral, and an informational interview in the same message.
 - **Ping once, then wait ~5-7 days** before a polite follow-up. Then stop; relationship over spam.
 - **Add value before you request value.** Comment on their posts, share a relevant report, or offer a 2-line insight — then ask.
 - **Track your outreach.** A simple sheet with name, date contacted, response, and next action will tell you which subject line and which role hook actually convert.
-

Closing Note

Your LinkedIn profile is a living document, not a one-time chore. Every time you earn a certification, finish a project, or notch a new number, that edit is a reason the algorithm re-shows you and a recruiter re-reads you. Keep it current, keep it quantified, and keep the healthcare keywords loaded — and your profile becomes the strongest job-search asset you own.

— *Built for HCA Daily career accelerators.*

Part 3 — The 30-Day Job Search Sprint

For Healthcare Administration Job Seekers

From Dr. Ashley Hussain-Okorafor, DBA

You are not going to "look for a job." You are going to run a 30-day sprint — a structured, measurable, no-excuses campaign that treats job searching like the project it is. Scattered effort got you scattered results. This is the opposite.

This plan is built for one person, fully self-serve, using tools you already have (a resume, LinkedIn, a spreadsheet, and 60–90 minutes a day). No calls, no coaches, no waiting. Just you, a plan, and 30 focused days.

The Rules of the Sprint

Print these. Tape them to your monitor. Break any of them and the sprint loses its power.

1. **Apply to 5 jobs per day.** Not "a few." Not "when I feel ready." Five. Every Monday through Friday. Weekends are for prep and rest.
2. **Tailor every single application.** Zero mass-applies. Each application gets a targeted resume (one page, rewritten for that role) and a custom 3–4 sentence cover note. Quality over quantity — but both.
3. **Follow up by Day 7.** Seven calendar days after an application with no response, send a polite check-in. One message. Then stop. Do not nag.
4. **Respond to every recruiter within 24 hours.** A fast reply is a competitive advantage. Block 15 minutes every morning for this.
5. **Track everything.** If it isn't in your tracker, it didn't happen. Numbers are the only honest mirror in a job search.
6. **Practice 15 minutes a day, every day, from Day 9 onward.** You cannot wing an interview in healthcare administration. You will rehearse.
7. **No social-scrolling masquerading as networking.** Connecting with strangers on LinkedIn is not work. Sending a targeted message to a hiring manager is.

8. **Take Sunday off.** One full day. Rest is part of the strategy, not a luxury. A burned-out candidate loses interviews.

The math: 5 applications/day × 20 working days = **100 quality applications** in 30 days. In healthcare administration, 100 tailored applications will convert at a realistic 3–8% to interviews. That's 3–8 interviews. That's an offer. Do the math daily and you will stop panicking.

Your Time Budget (per working day)

| Block | Time | Task |

|-----|-----|-----|

| Morning | 15 min | Respond to recruiter emails, check tracker, confirm today's 5 targets |

| Focus block | 75 min | Tailor and submit 5 applications (15 min each) |

| Midday | 15 min | One networking touch (DM, referral ask, alumni contact) |

| Evening | 15 min | Daily review, log statuses, prep tomorrow's list |

| **Daily total** | **2 hours** | — |

WEEK 1: The Foundation

Days 1-7 — Build the machine before you fire it.

Week 1 principle: *You are building tools you will use for the next three weeks. Do this right and every application, interview, and negotiation gets easier. Rush this and you bleed time all month.*

Day 1: Resume — Version 1.0

Goal: A clean, one-page, healthcare-administration-ready resume.

- [] Open your current resume and rewrite it from scratch — you're not editing, you're rebuilding.
- [] Use a single-column, reverse-chronological format. No graphics, no photos, no tables. Applicant tracking systems (ATS) must be able to read it.
- [] Every bullet follows the **Result Formula:** *Action verb + specific task + quantified result*. Example: *"Reduced patient pre-registration time by 22% by streamlining the intake workflow across three clinics."*
- [] Put a **Summary** line up top (2 sentences) naming your target role and your top 2 differentiators — don't make the reader guess who you are.
- [] Cut anything older than 10 years or unrelated. One page. Hard limit.
- [] Save as `FirstName\_LastName\_Resume\_HealthcareAdmin.docx` (Word, not PDF, for the ATS—you'll export a PDF later).
- [] **Action:** Extract 5 bullets from your current resume that are missing numbers and rewrite each with a metric. Put this metric bank in a scratch document — you'll build on it tomorrow.
- [] **Evening check:** Did every bullet have a verb and a number? Does the resume fit one page?

Day 2: Build Your Metric Bank

Goal: A running list of every accomplishment you've ever quantified — your raw material for tailoring.

- [] Create a document (or spreadsheet tab) titled **Metric Bank**.
- [] Mine your work history, internships, and volunteer roles for numbers in FIVE categories:
 1. **Patient outcomes:** satisfaction scores, wait times, readmission rates
 2. **Operations:** volume processed, time saved, steps eliminated
 3. **Money:** cost saved, revenue added, budget managed
 4. **Quality/compliance:** error reduction, audit results, accreditation
 5. **People:** team size, hires made, trainings delivered, retention improved
- 6. [] For each metric, write the **format:** *Action + quantity + context*. Example:
"Reduced patient wait time by 18 minutes in the orthopedic clinic by reallocating check-in staff."
- 7. [] Target: **15-20 banked metrics**. If you're early-career, use intern numbers, school project data, and volunteer clinic figures — they count.
- 8. [] **Evening check:** Do you have at least 10 metrics you could quote in an interview? If not, keep mining until you do — this is your competitive fuel.

Day 3: Your Target Role List (10+ Titles)

Goal: A precise list of the roles you're hunting — specific, real, and taunting.

- [] Pick **10-15 healthcare administration job titles** you will actively apply to. Mix in reach, match, and safety roles.
- [] Copy these into your tracker as your **watchlist** and set 3 active searches (Indeed, LinkedIn Jobs, hospital career portals).
- [] Here are 12 real, current titles to get you started — circle the ones that fit your experience level:
 1. Practice Manager / Medical Office Manager
 2. Clinic Administrator / Clinical Operations Manager

3. Patient Access Manager
4. Revenue Cycle Analyst
5. Utilization Review Coordinator
6. Healthcare Administrator (Practice / Multi-site)
7. Patient Services Manager / Patient Experience Manager
8. Billing & Coding Supervisor / Reimbursement Manager
9. Credentialing & Contracting Specialist
10. Health Information Management (HIM) Coordinator
11. Clinic Operations Coordinator / Administrative Coordinator
12. Population Health Coordinator / Care Coordination Manager

- [] For each title you choose, note the **top 3 requirements** recruiters list (from the posting) into your tracker. You'll mirror those exact keywords in your resume and cover note.
 - [] **Evening check:** You have a watchlist of 10+ specific titles and the searches to match. If you only have 3 titles, broaden your net tonight.
-

Day 4: LinkedIn — Engineer Your Profile

Goal: A profile that recruiters find and immediately trust.

- [] **Headline:** Not "Seeking opportunities." Use your title + differentiator. Example: *"Practice Manager / Patient Access & Revenue Cycle / AMB Reduction Strategist."*
- [] **About (150-200 words):** First 2 lines state your target role and value. Use keywords from your watchlist. No "I'm a hard worker" — show outcomes with your metric bank.
- [] **Experience:** Mirror your resume bullets, with your top 3 metric-driven bullets under each role.
- [] **Featured section:** Pin your polished HCA resume (PDF) and any certifications.
- [] **Open to Work:** Turn it on for *Recruiters only* (not public) if you prefer discretion, and set your target roles.
- [] **Headshot:** Bright, professional, front-facing. Crop tight. One photo change can cut your "how they look at you" risk in half.

- [] **Skills section:** Add the 5-10 skills your target roles list most frequently (e.g., *Epic EHR, Revenue Cycle, Patient Experience, HIPAA, Staff Scheduling*).
 - [] **Evening check:** Would a recruiter understand in 5 seconds what you do and for whom? Yes = done, no = rewrite the headline.
-

Day 5: Build Your Application Tracker

Goal: Your single source of truth. No memory, no sticky notes.

- [] Open a spreadsheet (Google Sheets or Excel) and copy the exact template below. One row per application.
 - [] Add **conditional formatting**: highlight "Applied" green, "Interview" blue, "No response past 7 days" yellow (needs follow-up), "Rejected" gray.
 - [] Set up a **calendar reminder** at 8 AM Monday-Friday: "Review tracker — 5 applications due today + follow-ups past Day 7."
 - [] **Evening check:** Tracker has all columns, formatting works, reminder is set. Test it by logging the 3 applications you'll submit tomorrow.
-

Day 6: Write Your Two Scripts (Cover Note + Networking)

Goal: Copy-paste-ready templates you customize in 3 minutes.

- [] **Cover note script (3-4 sentences):** Hook (one metric or achievement) → why this org (1 line) → why you for this role (mirror 2 requirements from the posting) → close ("I'd welcome 15 minutes to discuss how I can support your clinic's patient-access team."). Save this as a doc.
- [] **Networking message script (75-90 words):** Mutual point → specific compliment about their work → one relevant line about you → a small, specific ask (not "call me," but "could I ask a 10-minute question about your revenue-cycle path?"). Save as a second doc.
- [] Run a **test application** end-to-end: pick a real posting, tailor your resume + cover note, apply, log it. Time yourself — you're building your 15-minute-per-application rhythm.

- [] **Evening check:** Both scripts exist and fit their roles' word counts. You've submitted 1 practice application.
-

Day 7: Week 1 Review — The Foundation Is Built

Goal: Confirm the machine is ready, then rest.

- [] Re-read your resume. Read it out loud. Fix any phrase you stumble over.
 - [] Confirm you have: metric bank (15+ items), watchlist (10+ titles), clean LinkedIn, working tracker, two scripts.
 - [] Review your 5 test applications — did they log correctly? Fine-tune formatting.
 - [] Name your **top 3 target companies** and their career-portal URLs. Save them in your tracker as a "Companies" tab.
 - [] **Sunday:** Take the day off. Walk, sleep, eat well. Week 2 is a volume week.
-

WEEK 2: Applications — Volume + Precision

Days 8-14 — Five a day, each one tailored.

***Week 2 principle:** Volume without tailoring is spam. Tailoring without volume is hiding. You do both, every single day.*

Day 8: Qualify Today's 5 Targets

Goal: Five postings you genuinely want, pulled from your searches and company portals.

- [] Spend 30 minutes pulling the day's 5 postings from your watchlist searches + top-3 company career pages.
- [] Skip postings that are clearly a stretch on hard requirements (license, years). "Reach" roles are fine daily, but cap at 1 per batch of 5.
- [] Log all 5 into your tracker now (before you even write them) with a `Qualified` status.
- [] **Evening check:** 5 postings logged and matched to your watchlist titles. If a target company is thin, broaden the geography by 20 miles — you're building pipeline, not perfecting taste.

Day 9: The 15-Minute Application Protocol

Goal: A repeatable rhythm that produces 1 tailored application every 15 minutes.

For each of today's 5 applications, run this 15-minute protocol:

- **Minutes 1-2:** Read the posting. Underline 3-5 requirements and any repeating keywords.
- **Minutes 3-8:** Rewrite your one-page resume for THIS role — swap in your 3 strongest matching bullets from your metric bank, and mirror the posting's exact keywords (e.g., "patient experience," "EPIC", "CMS compliance").

- **Minutes 9-12:** Write a 3-4 sentence cover note from your script, hard-mirroring 2 posting requirements.
 - **Minutes 13-15:** Submit. Export the resume as PDF, upload your files, copy the posting's URL into your tracker, and set status to `Applied` with today's date.
 - **Timezone tip:** Apply between 8-10 AM or 1-3 PM local recruiter time, and always on the same day you find the posting. Early-mover advantage is real.
 - [] **Evening check:** Exactly 5 applications marked `Applied` today. If you only finished 3, finish 2 tomorrow before touching new postings.
-

Day 10-13: The Volume Streak

Goal: 5 tailored applications every single working day. Consistency, not bursts.

- [] **Day 10:** Submit 5. Pick 1 to send directly (not through the portal) to a hiring manager or director if their email is findable — use your networking script as the cover message.
 - [] **Day 11:** Submit 5. Refresh one stale search (e.g., swap "Practice Manager" for "Clinic Operations Manager") to surface new postings.
 - [] **Day 12:** Submit 5. Log any responses so far and run the **7-day follow-up check** (see Day 14).
 - [] **Day 13:** Submit 5. Start the interview prep habit (Week 3 begins officially tomorrow, but get 15 minutes of STAR rehearsal in tonight).
 - [] Each evening: update every row's status, zero unqualified postings sitting in limbo (reject or apply — no "maybe" status).
-

Day 14: Follow-Up Checkpoint + Week 2 Review

Goal: Clean the pipeline, honor the 7-day rule, prep for interviews.

- [] **Run the follow-up list:** Pull every application submitted 7+ calendar days ago with no response (rule #3).
- [] Send each one a single, short check-in (~90 words): gentle re-reference to the role, 1 line of why you fit, and "I'd be glad to answer any questions. Please let me know if you need anything further." Once sent, mark them `Followed up` and stop.

- [] Tally your Week 2 numbers. Log them: applications sent, response rate (responses ÷ applications), interviews secured.
 - [] **Sunday:** Rest. But set up your Week 3 space: print your job description, prep your interview tool kit (see Day 15).
-

WEEK 3: Interviews — Practice Makes Offers

Days 15-21 — Convert applications into confident conversations.

***Week 3 principle:** Interviews are a skill, not a personality test. You will rehearse until the answers are automatic.*

Day 15: Your Interview Tool Kit

Goal: Every answer resource in one place.

- [] Pull the full job description of your top target role. Highlight its 5 most repeated competencies.
 - [] Create a **competency map**: for each of the 5 competencies, write 1 metric-driven story from your metric bank you can tell in 90 seconds.
 - [] Write your **elevator intro** (60 seconds): who you are, your 2 differentiators, your target role, 1 strong metric. Rehearse it until you can say it without reading.
 - [] List **5 questions to ask the interviewer** (e.g., "What is the single biggest operational challenge this clinic is facing right now?" and "How does your team measure patient-access success?"). Asking sharp questions is how you signal you belong.
 - [] **Evening check:** Competency map done, intro rehearsed, 5 questions written.
-

Day 16: STAR Method — Build Your Story Bank

Goal: 8-10 rehearsed answers using Situation, Task, Action, Result.

- [] Using the **STAR structure**, write out:
 - **Situation/Task:** 2 sentences — the context and what was on your plate.
 - **Action:** 3-4 sentences — *what you personally did*, with a verb.

- **Result:** 1-2 sentences — the quantified outcome.
 - [] Cover these 8 common healthcare-admin themes: a time you **improved a process, resolved a conflict, handled high pressure, managed a budget, worked with physicians, hit a compliance or quality goal, delegated a team, and recovered from a mistake.**
 - [] Every story ends with the **metric**. No metric was saved? Tie the result to a team/org outcome ("...which kept our clinic in the top quartile for patient satisfaction that quarter").
 - [] **Evening check:** 8 of 10 stories written and metric-backed. These live in your Metric Bank as a "STAR Stories" tab.
-

Day 17: The AI Coach Practice Loop (15 min)

Goal: Real reps against real questions, with feedback.

HCA Daily's AI Interview Coach is built exactly for this. If you don't have a coach tool handy, use any AI chat as a mock interviewer — the loop is what matters:

1. **Prep (2 min):** Open your STAR story bank.
 2. **Drill (8 min):** Ask the coach a real behavioral question (e.g., "Tell me about a time you improved patient flow"). Answer it out loud in STAR format in under 90 seconds.
 3. **Feedback (3 min):** Ask the coach: *"Score this answer on structure, specificity, and metrics. Give me one concrete fix."* Note the fix.
 4. **Re-do (2 min):** Deliver the improved version once more, out loud.
 5. Repeat with 2 new questions tomorrow. **Daily target: 2 questions, 15 minutes, one recorded fix.**
-

Day 18-20: Interview Cadence — Ramp It Up

Goal: More reps, more variety, real prep for any live interviews landing.

- [] **Day 18:** Run 3 behavioral drills (15 min each block) — patient flow, conflict with a physician, exceeding a budget.
- [] **Day 19:** Run 3 more drills + 2 "hardball" questions (gaps in experience, "why leave your current role?") — answer honestly, briefly, and reframe to what you *do* bring.

- [] **Day 20:** Dry-run a **full 30-minute mock interview** with the coach covering 6 questions, then debrief: what was your weakest answer?
 - [] If you have real interviews scheduled: **15 minutes of prep the night before** (re-read the JD, competency map, 5 questions) and **15 minutes the morning of** (elevator intro + 2 most-likely stories).
 - [] Send **thank-you notes within 24 hours** of every interview — a 3-sentence email: thank them, restate 1 strength you bring, reference 1 specific point from the conversation.
-

Day 21: Week 3 Review

Goal: Log results, harden your strongest stories, rest.

- [] Count interviews secured and completed this week.
 - [] Re-drill your **top 5 most-likely stories** until they're effortless.
 - [] Confirm every interview gratitude note is sent.
 - [] **Sunday:** Full rest day. Offers and next steps come in Week 4 — show up fresh.
-

WEEK 4: Follow-Up + Offers

Days 22-30 — Close the pipeline and negotiate from strength.

***Week 4 principle:** The job search doesn't end at "yes." It ends when you've negotiated a fair deal and handed off cleanly. You will not leave money on the table — not when you've come this far to get the offer.*

Day 22: The Follow-Up Sweep

Goal: Activate every dormant application one final, respectful time.

- [] Pull all applications still in `Applied` or `Followed up` with no response past 10 days.
 - [] Send a **final, polite nudge** (~80 words) referencing the original role, your continued interest, and a closing line ("I'd love to know if the timeline has shifted"). One message. Then let it go — keep it warm, not desperate.
 - [] For any recruiter or hiring manager you had a live back-and-forth with and haven't heard from in 5 days, send a short "touching base" note.
 - [] **Evening check:** Every stale row now has a status. Nothing is silently sitting.
-

Day 23: Negotiation Prep — Know Your Number

Goal: Know your worth and your walk-away before any offer arrive.

- [] Research market pay for your target role in your metro using BLS (bls.gov) healthcare-administration categories and pay-transparency ranges in postings.
- [] Pick your **target salary** (realistic high end) and your **floor** (absolute minimum to accept, factoring in commute, benefits, and growth).
- [] List your **3 strongest negotiating chips** — quantified achievements that justify above-midrange (e.g., "reduced AMB denials 18%," "cut intake wait time 22%").

- [] Practice saying the ask: "**Based on my range and where I'd add value, I was expecting something in the \$X to \$Y range. Is there flexibility?**" — say it out loud until it's comfortable.
 - [] **Evening check:** Target, floor, and chips written and rehearsed. You can say the number without flinching.
-

Day 24-26: Offer Handling Drill

Goal: Know exactly what happens when an offer lands — so you don't freeze.

- [] **Day 24:** Rehearse how you'll respond to a verbal offer, by phone or email — you never accept on the spot. Script: *"Thank you, I'm excited about this. I'd like to review the full details and get back to you within [timeframe]."* Rehearse the alternative — *response to a too-low first offer* (ask a question, don't react: "Given my experience with [metric], how flexible is that range?").
 - [] **Day 25:** Build your **offer checklist** — review total comp (base, bonus, sign-on, benefits, PTO, remote/hybrid, start date). Nothing is accepted until every box is confirmed.
 - [] **Day 26:** Practice the **full negotiation script** back-to-back with the coach until you can run both the accept path and the ask-for-more path comfortably.
 - [] **Evening checks:** You could receive an offer and respond calmly, professionally, and without over-committing.
-

Day 27-29: Closing & Decision Protocol

Goal: Convert an offer into a sign, or a fast re-route if none lands yet.

- [] **If you have an offer:** Run your checklist, make the counter (if warranted), confirm the start date, and get written confirmation of the full package before you resign anywhere. Notify competing employers respectfully — a polite "I have an offer but you remain my first choice if you can move quickly" can force a better deal.
- [] **If you don't have an offer yet:** Do NOT panic. Your pipeline is warm. Commit to **5 more tailored applications per day** for these 3 days; interviews take 1-2 weeks to surface, so volume continues to fuel the funnel. Re-run your 15-minute protocol every morning.

- [] **Day 28:** Deep-review your top 3 pending interviews or applications — send a strategic reintroduction or ask a mentor/alumni contact to refer you (a referral multiplies your odds of an interview call).
 - [] **Day 29:** Final full review of your tracker. Calculate your month's numbers honestly.
-

Day 30: Sprint Review — Your Numbers Tell the Story

Goal: Close the loop, bank the win, restate your path forward.

- [] Complete your **month-end numbers sheet**:

| Metric | Your Target | Your Actual |

|-----|-----|-----|

| Applications submitted | 100 | |

| Applications followed up | All due | |

| Interviews received | 3-8 | |

| Thank-you notes sent | All | |

| Networking touches | 15+ | |

| Offers received | 1+ | |

| Offer accepted | — | |

- [] If you accepted an offer: congratulations — record the full package, thank everyone who helped, and update your LinkedIn (announce from a place of humility and gratitude).
- [] If the sprint runs on (jobs take time!): you are not starting over. You have a resume machine, a metric bank, a warm pipeline, and rehearsal reps. Re-run Week 2 volume days at 10/day with the same tools until you close.
- [] Take today to celebrate the work. You ran a disciplined campaign. That discipline is what gets hired.

Your Application Tracker Template

Copy this into a spreadsheet. One row per application. Columns are your contract with yourself.

| Date Applied | Company | Role Title | Source | Posting URL | Resume Tailored? | Cover Note? | Status | Follow-up Day 7? | Next Action | Notes |

|---|---|---|---|---|---|---|---|---|---|

| 2026-09-13 | St. Mary's Health | Patient Access Manager | Their career portal | [url] | Yes | Yes | Applied | Due 09-20 | Follow up 09-20 | Met director on LinkedIn |

| 2026-09-13 | Riverview Clinics | Practice Manager | LinkedIn Jobs | [url] | Yes | No | Applied | Due 09-20 | — | Includes bonus info |

| 2026-09-14 | Coastal Health | Revenue Cycle Analyst | Indeed | [url] | Yes | Yes | Interview | — | Prep night before 09-18 | Recruiter replied 09-15 |

|||||||

Status options: Qualified → Applied → Followed up → Interview → Offer → Accepted / Rejected / Closed.

Rule of thumb: If a row sits in `Qualified` or `Applied` past 7 days with nothing done, it's a follow-up trigger — then it becomes `Followed up` or gets closed.

Daily Checklist (copy for each working day)

- [] Responded to all recruiter emails within 24 hrs (15 min)
- [] Logged/confirmed today's 5 application targets in tracker
- [] Submitted exactly 5 tailored applications (75 min)
- [] Sent 1 networking touch (DM, referral, alumni note) (15 min)
- [] Ran a 7-day follow-up check; sent any due nudges
- [] Rehearsed 15 min of STAR/AI-coach practice (from Day 9 onward)
- [] Evening review: logged all statuses, prepped tomorrow's targets

Weekly Review (end of each week)

- [] Weekly numbers logged: sent, response rate, interviews
 - [] Pipeline clean (no silent limbo rows)
 - [] Top stories re-drilled (from Week 3 onward)
 - [] Sunday fully off
-

You've got a plan. The plan is the advantage. Now run the sprint.

— Dr. Ashley Hussain-Okorafor

Part 4 — The Salary & Offer Negotiation Masterclass

HCA Daily — Premium Career Product

By Dr. Ashley Hussain-Okorafor, DBA · Founder, HCA Daily

The companion product, the Interview Answer Vault, taught you how to answer the question. This masterclass teaches you how to win the room — and then win the number.

If you've been through the Vault's Part 5, you know the basics: never anchor first, negotiate the package, get it in writing. This is the deep-dive. Here we dissect the actual **anatomy of a healthcare-administration offer**, arm you with the **benchmark data recruiters and HR are literally reading from** (MGMA, BLS, ACHE), hand you **ten verbatim counter-offer scripts** with the behavioral psychology behind why each works, and walk you through the tough moments nobody likes to script — the lowball, the "best and final," and the offer that got pulled.

Settle in. Class is in session.

Part 1 — Total Compensation Anatomy

Read the whole package before you negotiate a single dollar

Here's the most common mistake high-performers make: they negotiate **base salary** and leave \$15,000-\$40,000 a year on the table in the other seven levers. In healthcare administration, base is frequently the most *rigid* number in the offer — frozen by pay bands, system structure, or comparability rules. The other components are where real money moves.

Below are realistic 2025–2026 ranges for the three levels you actually negotiate at. **Base ranges** are drawn from MGMA DataDive Management & Staff Compensation data, BLS Occupational Outlook median for medical & health services managers (\$117,960 in May 2024), and the ERI compensation database for medical practice administrators. The **add-on ranges** reflect what employers commonly approve for each level.

Component-by-component, by level

| Component | Practice Manager (single site) | Director / Multi-site / Practice Administrator | VP / Executive |

|---|---|---|---|

| **Base salary** | \$68,000 – \$95,000 | \$110,000 – \$180,000 | \$180,000 – \$305,000+ |

| **Sign-on bonus** | \$3,000 – \$15,000 | \$10,000 – \$50,000 | \$25,000 – \$100,000 (recruitment-challenged markets) |

| **Relocation assistance** | \$5,000 – \$15,000 | \$10,000 – \$30,000 | \$20,000 – \$60,000 (often grossed-up) |

| **Annual incentive / bonus target** | 5–10% of base | 10–25% of base | 15–40% of base |

| **PTO (total annual)** | 15–20 days | 20–25 days | 20–30 days + sabbatical/conference flex |

| **CME / conference allowance** | \$1,500 – \$2,500 | \$2,500 – \$4,000 | \$4,000 – \$7,500 |

| **Licensure / certification reimbursement** | \$500 – \$1,500 | \$1,500 – \$4,000 | \$3,000 – \$6,000 |

| **Retirement match** | 3–5% of pay | 4–6% of pay | 5–8% of pay (plus deferred comp at exec levels) |

What the data actually says (quote these)

- **BLS (May 2024):** The median annual wage for *medical and health services managers* was **\$117,960**. The lowest 10% earned under **\$69,680**; the highest 10% earned over **\$219,080**. The field is projected to grow **23% from 2024 to 2034** — more than four times the average — which is *your leverage*: demand is outpacing supply.
- **MGMA DataDive Management & Staff Compensation (2024 report):** Management compensation (executive, senior, general) rose **6.95%** in median total compensation in 2023, and executive management total comp is up **21.6%** over ten years. General management's 10-year growth was **29.4%**; senior management's was **31.2%**.
- **MGMA continuing-education data:** executive management positions average **\$4,000** in CME dollars and **28 PTO hours** for continuing education; senior and general management average **\$2,000** and **16 PTO hours**. Use this when you ask for "at least market CME."

- **MGMA regional spreads:** executive management pay differs by as much as **\$101,000 between the highest- and lowest-paying regions**; senior management differs by **~\$34,000**; general management by **~\$15,000**. West Coast, Northeast, and large metros run 20–35% above the Southeast/Midwest.
- **ERI (2026):** the average **medical practice administrator** earns **\$224K** (typical range \$150K–\$277K) — confirming that large-group administrator and VP-level roles comfortably clear \$200K.

The four hidden rules of total comp

1. **Incentive percent is as negotiable as base.** A VP asked for a 20% target and got 25%. A manager's bonus "target" of 5% can become 8% with an outcomes clause. The base band is often a hard wall; the **percent** is a dial.
 2. **Vesting is the quiet killer.** A \$50K sign-on paid over four years *and* tied to a 12-month clawback is effectively a retention bonus, not a signing bonus. Read the vesting schedule before you celebrate. (More in Part 6.)
 3. **CME + licensure + cert money advertises how they'll invest in you.** A \$1,500 CME budget and zero cert support tells you this employer expects you to stay sharp on your own dime. That signal matters for your growth trajectory — not just the dollars.
 4. **Retirement match compounds like it's the only number that matters.** At VP level, a 3% vs. 7% match on a \$250K base is a **\$10,000-per-year** difference. Over a five-year tenure at 7% growth that's roughly \$57,000 of extra retirement value. You should care about this more than \$5K of sign-on.
-

Part 2 — The Offer Evaluation Framework

Judge the offer like an analyst, not an optimist

You will be tempted to evaluate an offer on *feeling*. Resisting that is the whole game. Work this seven-step framework before you say yes to anything.

Step 1 — Convert everything to a single "Year-One Total Comp" number.

Add base + guaranteed sign-on (year-one portion after vesting) + relocation + expected incentive (target percent × base) + retirement match dollars + the *cash value* of extra PTO days beyond your current employer's.

Step 2 — Isolate what's guaranteed vs. what's at risk.

Base, sign-on, relocation, and match are near-guaranteed. Incentive is *at risk* (typically 60–80% of target is realistic, not 100%). Label line items "guaranteed" and "at risk" and run your Year-One Total Comp both ways — full target and 70% of target. Decide which number you can actually plan a life on.

Step 3 — Compare against market, not against your current paycheck.

Use the Part 1 ranges and your geography adjustment. The question isn't "is it more than I make now?" It's "*is it inside the band for this role in this market?*" A \$10K raise can still be a \$15K-under-market offer.

Step 4 — Price the intangibles in dollars.

Assign cash value to: remote days, commute time, PTO, on-call/weekend obligations, decision rights, title, a written 6-month review, and the CME/cert budget. A \$5K-lower base with 3 extra weeks of PTO and full cert reimbursement can be worth more in cash-equivalent terms *and* in trajectory.

Step 5 — Run the 12-month and 36-month scenario.

What does your comp look like at the 12-month market review? At the 36-month mark if you hit 100% of target? Projected earning power matters more for a 3-year commitment than the Year-One number.

Step 6 — Stress-test with three questions.

(a) *Could I get this in writing?* (b) *Is the sign-on clawback reasonable and incentive achievable (ask what % of people hit >70% of target)?* (c) *If I hit every goal and they're solvent, does this scale toward my 5-year number?*

Step 7 — Get competitive context before you sign.

If you're deciding without at least one other benchmark (a competing offer, a strong counter elsewhere, or a current-employer retention counter), you're negotiating blind. You don't need an offer *in hand* — but you need a realistic *number in mind* from the data and from your network. (ACHE and your professional association listserv are goldmines for current bands; use them.)

Decision rule: an offer is a **yes** when guaranteed Year-One Total Comp clears your walk-away number *and* the intangibles support your trajectory — not when it "feels right" at 11 p.m. on a Tuesday.

Part 3 — Ten Verbatim Counter-Offer Scripts

Say these. Every one has the psychology attached.

These are written as the literal words that come out of your mouth or your email. Fill the brackets, keep the warmth, and **read them until they're muscle memory before your first three negotiations**. Each script ends with the psychology of why it wins.

Script 1 — The reinvention: never say a number first

When to use it: First screen or first call, before the role is fully scoped. They ask, "What are your salary expectations?"

"Happy to talk about that — I want to be sure I'm giving you a real number and not a guess. Can you tell me the range that's been budgeted for this role? If it lines up with the market, I'm confident we'll land it. And I'll be straight with you: I'm looking for something that reflects the scope of this job, and I'm not going to price myself out of work I genuinely want."

Why it works: Whoever names the first number sets the anchor — and the anchor almost always drags the final number toward whichever side planted it. This frames the request as cooperation ("give me the range so I don't guess") rather than evasion, and forces *their* band onto the table first. If the band comes back "\$95K-\$120K," you now aim high *inside* that band instead of cold-anchoring at \$100K.

Script 2 — Anchoring high on your own terms (when you must go first)

When to use it: They've asked twice, won't share a band, and you have to name a figure.

"Based on the scope here — [NUMBER OF SITES / STAFF / FTEs] and the operational turnaround you described — and against what I'm seeing for comparable roles in [METRO/REGION], my target is [AMOUNT]. That's where I'd want to land. If the range can't reach it, I'm open to talking about the rest of the package — but I want to be honest with you about where I stand."

Why it works: You haven't anchored a random number; you've anchored a *justified* number tied to scope and market, which is far harder to talk you down from. You state it once, without apology, and then you go silent. The silence is the tool — the other side fills it, and usually with a number, not with the word "no." Never talk past your own anchor.

Script 3 — The "range problem or package problem" reframe

When to use it: You named a number and they say it's over budget, over band, or "above what we can do."

"I appreciate you being straight with me. Two questions so I can work with you: what's the top of the range, and is that a hard ceiling or is there flexibility for the right candidate? I'm not trying to break your structure — I'm trying to understand whether this is a range problem or a total-package problem, because those get solved differently."

Why it works: This converts a potential yes/no dead end into a *how* conversation. "Range problem or package problem" forces them to reveal the true ceiling (the number you then negotiate against) and signals you understand their internal constraints — which makes them want to help you find creative room. Cooperation beats confrontation, every time.

Script 4 — Trading base for package when the band is frozen

When to use it: They've told you the base pay band is a hard wall — system-wide policy, union step scale, or comparability rules.

"I hear you that the base is locked — I'm not asking you to break your structure. What I'd like instead is a one-time sign-on of [AMOUNT], relocation of [AMOUNT], an annual incentive target of [X]%, and a written market review at 12 months. None of those touch your pay band, and together they close the gap for me in year one. If we can land on that, I'm ready to sign today."

Why it works: You've solved their compliance problem *for them* — the person across the table now has a package they can defend internally without breaking a rule. You've bundled several negotiable levers so a "no" on one doesn't sink the whole ask. And

you've given the recruiter a closing line ("she's ready to sign today") they can sell to their boss. Prepared to sign is the single most powerful lever in any negotiation.

Script 5 — The principle-based "let's get close"

When to use it: You're \$5,000–\$20,000 apart on base and the conversation is healthy.

"We're not far apart, and I'd rather get this right than get hung up. If you can get the base to [AMOUNT], I'll match you by being flexible on [LESS-IMPORTANT ITEM, e.g. start date / incentive target]. If base can't move, then I'd want to close the gap with an early market review at six months, in writing, plus a higher sign-on. Which of those works for your side?"

Why it works: This introduces **reciprocity** — behavioral economics shows people feel compelled to give something back when you offer a concession. You've also given them a *menu*, so they choose the path that fits their constraints, which shifts them from resisting your request to *solving your request*. Option A (base) or Option B (six-month review + sign-on) — either way you win.

Script 6 — The six-month-review card (when base is truly dead)

When to use it: Base is absolutely fixed and you need a *tonnage* answer for your own financial planning.

"I understand the base is where it is. Here's what makes this workable for me: a written commitment to a market and performance review at [SIX/TWELVE] months, with the conversation starting at [AMOUNT], not from zero. I'd also like that review documented in my offer letter so it survives turnover in leadership. If we can put that in writing, this is a yes."

Why it works: A future-dated commitment has almost no cost to them today — which is exactly why "yes" is easy — but it has massive value to you. "Starting at [AMOUNT], not from zero" prevents the classic bait-and-switch where a 3% "market" raise arrives at review time. **Insisting it be in the offer letter** is the difference between a promise and a contract; promises evaporate when the hiring manager leaves.

Script 7 — Using a competing offer (without being a jerk)

When to use it: You have another offer OR a credible retention counter, and you want this employer to move.

"I want to be transparent with you, because I'd rather work here than anywhere else I'm talking to. I have another offer at [AMOUNT], and [EMPLOYER] has asked me for an answer by [DATE]. This job is my first choice — but I need the package to be competitive for me to be able to choose it. What can you do on [BASE / SIGN-ON / INCENTIVE] to bring us together?"

Why it works: You've reframed the situation from *demand* to a *choice you'd rather make in their favor* — which puts them in a position of *losing you* rather than *paying you*. Fear of loss is a far stronger motivator than hope of gain. And the deadline creates **scarcity with a clock**, which compresses their internal decision-making. The magic phrase is *"I'd rather work here"* — it lets them win while closing the gap.

Script 8 — The salary question from the employer's own comp analytics

When to use it: They ask, "Where are you currently, or what do you need to make this work?" — and you want a data-backed, non-confessional number.

"Market data for this role in [REGION] puts the range at [RANGE], and the position you've described has the scope of the upper part of that band. I'd target [AMOUNT] as the salary and I'm open to a flexible package that gets me to that in year one. Can we talk about what that looks like?"

Why it works: Your number is anchored in *public, shared data* (MGMA/BLS/ACHE), not your personal circumstances — which means it's not a negotiation against your past, it's a negotiation against *market reality*, and they can't argue with published benchmarks the way they can argue with an "I need more" request. It also short-circuits the "what do you currently make?" trap by answering about the *role's market value* instead.

Script 9 — The graceful package remodel when salary truly can't move

When to use it: You're willing to accept a lower base because this is your dream role or the trajectory is worth it — but you're not willing to leave money on the table.

"I'm comfortable with the base as offered because this role and team are the right step for me. What I'd ask for instead is to reshape the package: an extra week of PTO, a higher CME and certification budget, and a committed path to [NEXT CERT / LEADERSHIP ROLE] within eighteen months. If we can put the review cadence and the development commitment in the offer letter, I'm ready to sign."

Why it works: You've made *conceding on the base* look like a smart, confident choice to invest in the right long-term role — which is exactly the self-image professionals want to project. And you've converted the concession into a *measured package upgrade plus a documented development plan*, which (a) preserves your cash-equivalent value and (b) forces the employer to write down a growth commitment they'll be accountable for later. Strong candidates get paid in trajectory when they can't get paid in cash — this is how you *make sure the trajectory actually arrives*.

Script 10 — The pre-signing "make it formal" closer

When to use it: You've verbally agreed and now must get the final number in writing before you give notice anywhere.

"This is exactly what I was hoping for — I'm excited to join. Before I give notice to my current employer, I need the final offer in writing with the base, sign-on, incentive target, and review terms all documented. I know you'll make good on it, but I need to be able to show my family and my current employer a document, and I want us both to be protected. Can you send the formal offer today?"

Why it works: This protects you from "recruiter enthusiasm" — verbal numbers that quietly change on the written offer. It's framed as *protecting both of you* and as a *practical necessity* (you can't responsibly resign without documentation), not as distrust. And it creates a **timing contract**: they know the written offer closes the deal, so they're motivated to move fast while your enthusiasm is at its peak.

Part 4 — Negotiating a Sign-On Bonus When the Base Is Capped

The single highest-leverage move in healthcare administration hiring

When base is frozen (pay band, union step, system-wide comparability, "no exceptions" policy), the **sign-on bonus is your escape valve** because it *doesn't touch the pay band at all*. Here's the full playbook.

The reframe that unlocks the bonus: Every employer has two budgets — the *salary budget* (rigid, governed by bands and structure) and the *recruitment/incentive budget* (flexible, often zeroed out at year-end). Your job is to move your ask from the frozen account to the fluid one. Say it plainly:

"I understand the base is locked, and I'm not going to ask you to break the structure. What I'd like is for the gap to be closed as a one-time sign-on bonus, since that doesn't affect your pay band or your ongoing salary structure at all."

Make the number concrete and defensible. A sign-on is approved far more easily when it's framed against a real cost, not a wish:

"Based on my market research, hiring a practice manager who can also own the revenue-cycle improvement means at least [SIX/EIGHT] weeks of search and onboarding before anyone's fully productive. A sign-on of [AMOUNT] is a rounding error against that. It closes the gap for me, and it costs your team nothing in ongoing payroll."

Now the structure — vesting is where deals get poisoned. A \$40,000 sign-on paid over four years, with a 12-month clawback if you leave, is a *retention bonus* wearing a sign-on costume. Ask explicitly:

"A couple of clarifying questions so we don't misalign: how is the sign-on paid — all upfront or in installments? And if I leave, is there a clawback, and over what period? I want to plan my finances on the real number, not the headline."

What strong candidates actually get:

| Lever | What to ask | Realistic outcome |

|---|---|---|

| Upfront portion | 50–100% in first paycheck | Many systems pay 100% upfront; if installments, push for a large first tranche |

| Clawback period | Ask for 12 months or less | 6–12 months is fair; 24–48 months is a trap — negotiate it down |

| In the offer letter | The exact amount, payment timing, and clawback terms in writing | Non-negotiable requirement — never accept a verbal "\$X sign-on" |

| Gross-up | Ask if taxes are grossed-up | Some employers gross-up relocation, fewer gross-up sign-ons — but you don't get what you don't ask for |

Bundle it so "no" is never fatal. One ask that fails can sink a deal; three or four levers mean a "no" on one is survivable. Pair the sign-on with a six-month review, an extra PTO week, and a CME bump. You'll walk away with *something* every time — and usually more than you'd have gotten fighting for base.

The closing condition (decisive, always):

"If we can land on [AMOUNT] sign-on plus the 12-month review in writing, I'm ready to sign today."

Part 5 — Timing Rules: When to Talk Numbers, Who to Talk To

The golden rule: *Never negotiate money before they've made an offer.* Before that point you're a candidate being screened; after that point you're a *chosen* candidate they've begun to emotionally and logistically commit to. Leverage is maximized once they've chosen you. Everything before the offer is about **scoping** the role and **collecting** their constraints — not about naming your number.

When the salary question comes early, deflect (Script 1), then re-time:

"I'd rather talk numbers in detail once we're sure this is the right fit on both sides. Can I ask what the band is so I know we're in the same zip code — and understand the scope a bit more first?"

Who to talk to — and the levels matter:

| Contact | What they can and can't do | Your move |

|---|---|---|

| **Recruiter / HR screener** | Almost no authority over the offer; but they control the *range reveal* and your *screening outcome* | Stay warm, deflect the anchor, get their band, never negotiate the final number with them |

| **Hiring manager (Director/VP you report to)** | Owns the *scope* and can fight for you internally; usually not the comp authority, but their endorsement is what unlocks exceptions | Build your value case; they're your *internal advocate* — give them the argument to take up the chain |

| **Compensation / HR leader** | Holds the pay-band and exception authority | This is where the actual money decisions get made — bring the market data |

| **Finance/administrator** | Approves sign-on, relocation, incentive structure | Your sign-on and incentive asks live here; frame them against cost-of-hire |

When to escalate: If the person you're talking to keeps saying "I can't approve that," *thank them, and ask who can* — then offer to make the case directly:

"I completely understand that's above your level. Who on your team would be able to approve a sign-on exception, and is it appropriate for me to make the case to them directly? I'm happy to do the homework to help you get it approved."

Three timing traps to avoid:

1. **Never resign before the written offer.** Verbal agreement + notice = you've volunteered all your leverage and set yourself up for a rescission. (Part 6 handles this.)
2. **Don't negotiate against silence.** If they say "we'll get back to you," don't fill the gap with a worse offer to yourself. Reiterate your excitement and your ask, then wait.
3. **Don't let an offer age past its warmth.** Reply to offers within 24-48 hours to keep momentum and your leverage at full strength.

Part 6 — The Three Hard Situations, Scripted

Lowball, Best & Final, and Rescinded Offers

Situation A — The Lowball

A lowball isn't an insult; it's a *test* — and frequently the opening bid, not the final one. Your job is to stay warm, stay process-oriented, and see whether it's a *bottom-feeding opening* or a *genuine ceiling*.

Step 1 — Never say no on the spot. Say this:

"I appreciate you putting that forward. That's below the market range I shared for a role with this scope in [REGION] — where the position sits is [AMOUNT] higher. I want to be sure we're not anchored in the wrong place. Can you tell me how that number was built?"

Why it works: "How was that number built?" forces them to *justify* their own low offer, which weakens it in their own mind and reveals whether it's a structure or a test. You've declined without being hostile, and you've kept the door open.

Step 2 — If it's structurally low, reframe up without disrespecting them:

"I get that you may have budgeted for a more entry-level profile. If that's the ceiling for this role, I understand — but it's not a match for the level of scope you've described. Is there flexibility in the package to get to market value, or is this role truly banded at [LOW AMOUNT]? Because that would change the type of role this is."

Step 3 — Have a walk-away and mean it. If it's genuinely a ceiling and they won't move, thank them, express enthusiasm to "keep in touch for future leadership roles," and *walk*. Walking from a real lowball that can't move is not failure — it's the data telling you this employer values the role below what you'll ever be happy earning there.

Situation B — "This is our best and final"

"Best and final" is often a pressure tactic, not a last word. But you must treat it with *respect* because calling a false bluff can backfire. The pro move: **get everything on the**

table, in writing, before you respond — and don't spend your final ask on a trivial item.

Say this:

"Thank you — I appreciate you being decisive, and I'll be the same. Before I give my answer, can you confirm that [the base, sign-on, incentive target, and vesting terms] are all final as stated, in writing? If that's truly the best, then I need a beat to decide — and if there's any room left in the package, now is the time to put your strongest offer forward, because I'd love for this to be a yes."

Why it works: You've (a) verified there's no hidden room ("now is the time to put your strongest forward"), (b) *forced the written terms* rather than responding to a verbal pressure point, and (c) kept the door open on a positive note so you're not burning the bridge if they do hold firm. If it's genuinely final, you now decide on *accurate* information. And if they had any remaining headroom, the deadline has just extracted it.

Situation C — The Rescinded Offer

When an offer gets pulled — after a hiring freeze, a budget review, a manager change, or (most painfully) after you've resigned — do **not** spiral, and do **not** burn the bridge with the same side. Handle it in three beats:

Beat 1 — Immediately, calmly, in writing (anything verbal, follow up in email):

"Understood. I'm disappointed, of course — and I know these decisions often sit above everyone I've worked with. I'd genuinely value keeping this relationship intact. Given that I've been planning around this, is there anything you can do — a freeze-and-revisit in [X] weeks, a lateral role in the same system, or a formal recommendation I could route to peer organizations in your network?"

Why it works: Employers who rescind feel *guilt*, and guilt is a lever. By naming the disappointment *while* offering an easy path back, you leave room for them to reopen the door, place you elsewhere in the system, or hand you referrals — all of which are worth more than a bitter email.

Beat 2 — Call your recruiter/HR contact who's *not* the one who rescinded. Ask for a candid read:

"Can you tell me, off the record, whether this is permanent or a freeze-and-revisit? And would it be okay to leave the offer on the table to revisit in [X] weeks — or have you already been through that process with similar roles?"

Beat 3 — Convert to optionality, immediately. Rescissions are rarely about you. Reactivate your pipeline, notify your reference network ("my timeline just opened in a way I didn't plan"), and treat the *competing interest you generated* as your real insulation. The candidate who never stopped generating options is never devastated by a rescission.

Prevention (the part that actually matters): Insist on the written offer *before* you resign (Script 10) — it won't guarantee the offer survives, but it dramatically reduces the odds and gives you contractual recourse and a much stronger negotiation position if it does get pulled. **Never resign on a handshake.**

Part 7 — The Negotiation Checklist

Work this list on every offer, in order. If you can honestly check every box, you've negotiated like a professional.

Before you engage:

- [] I know the market range for this role in my region (MGMA/BLS/ACHE) and my geography adjustment.
- [] I have a written **walk-away number** — the Year-One Total Comp below which I walk.
- [] I have a **target number** (anchor) set ~10–15% above what I'd happily accept.
- [] I've identified my three competing benchmarks or at least one live alternative.
- [] I know who actually holds the comp authority on this role.

During the conversation:

- [] I never named a number first unless forced (and then I anchored high, justified, silent).
- [] I negotiated the **package**, not just base — sign-on, relocation, incentive %, PTO, CME, cert, match, review timing.
- [] Every "no" on one lever was answered with a different lever, not with giving up.

- ☐ I used market data (MGMA/BLS) as my justification, never personal need.
- ☐ I stayed warm and process-oriented the entire time (no threats, no pleading).
- ☐ I asked how the number was built when the offer felt low.
- ☐ I treated "best and final" as real, but verified all terms and terms-in-writing first.

Before you sign:

- ☐ I have the **final offer in writing** — base, sign-on, payment timing, clawback, incentive target, and review terms all documented.
- ☐ I have read the **vesting schedule** on my sign-on and match and understand my real year-one value.
- ☐ I've run Year-One Total Comp both at full-target and 70%-of-target.
- ☐ I've confirmed the written review commitment starts at [AMOUNT], not from zero.
- ☐ I have NOT resigned from my current role based on a handshake.
- ☐ I've made a yes/no/walk decision against my walk-away number, not my feelings.

Sign and launch:

- ☐ I've documented the offer and will send a short, grateful confirmation email restating the key terms.
- ☐ I've set calendar reminders for my 6- and 12-month market reviews so the commitment can't expire silently.
- ☐ I've noted my new CME/cert budget and committed to using it within the first year.

Sources & Further Benchmarking

- **U.S. Bureau of Labor Statistics, Occupational Outlook Handbook** — medical & health services managers: median \$117,960 (May 2024); 10th percentile <\$69,680; 90th percentile >\$219,080; 23% projected growth 2024-2034.
- **MGMA DataDive Management & Staff Compensation Data Report (2024 & 2025)** — base/variable comp, bonus/incentive, retirement benefits, PTO, and continuing-education benchmarks across general/senior/executive management; regional spreads (executive management differs by up to ~\$101K across regions).
- **MGMA DataDive Management & Staff Compensation Finder / Salary Calculator** — role-, region-, and credential-filterable compensation benchmarks.

- **ERI Economic Research Institute** — 2026 medical practice administrator compensation (avg ~\$224K; typical \$150K-\$277K).
- **American College of Healthcare Executives (ACHE)** — career resource center, compensation surveys, and credential (FACHE) support resources.

Reminder: ranges vary by cost of living, payer mix, organization size, and ownership model — always sanity-check against the live MGMA finder for your specific region and setting before you anchor.

End of the Masterclass. You now negotiate with data in one hand and scripts in the other. Go get the number you're worth — and remember: the goal isn't to win a negotiation, it's to get fairly paid for the work healthcare administration deserves to have done well.